



# Oral Surgery Third stage

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# Lecture 1

## Case sheet



# Intended learning outcomes

At the end of the lecture you will be able to :

- Illustrate the importance of correct diagnosis
- Explain history taking
- Identify the objectives of history taking
- Tell how to take history
- Summarize the components of history taking



# Oral and Maxillofacial surgery

Is one of the dental specialties dealing with management of diseases, injuries and defects of human jaws and associated structures. Oral surgery forms the connecting link between medical and dental specialties.



# Diagnosis in surgery

Oral diagnosis is the art of using the **scientific knowledge** to identify the oral diseases and also to distinguish one disease from another. The diagnostic process classically involves the following steps:-

# Diagnosis in surgery

The diagnostic process classically involved the following steps-:

- ✓ History taking.
- ✓ Clinical examination.
- ✓ Investigations.
- ✓ Provisional diagnosis.
- ✓ Definitive diagnosis and treatment plan.



Oral Surgery  
Case Sheet

Date: \_\_\_\_/\_\_\_\_/\_\_\_\_

| Student information |             |  |  |
|---------------------|-------------|--|--|
| Name                | Stage/Group |  |  |

| patient information |       |            |  |
|---------------------|-------|------------|--|
| Patient's Name      | Age   | Gender     |  |
| Address             | Tele. | Occupation |  |

| Chief complaint (CC) |         |  |
|----------------------|---------|--|
| Pain                 | Others: |  |
| Swelling             |         |  |

| History of Present Illness (HPI) |       |                 |              |          |                  |
|----------------------------------|-------|-----------------|--------------|----------|------------------|
| Pain site :                      |       |                 |              |          |                  |
| Pain nature:                     | dull  |                 | continuous   |          | tender           |
|                                  | sharp |                 | intermittent |          | not tender       |
| Pain onset :                     | day   |                 | Night        |          | Non specific     |
| Pain duration:                   |       | Trigger factors |              |          | Pain persistence |
| Swelling                         |       |                 |              | Duration |                  |
| Others:                          |       |                 |              |          |                  |

| Past Dental History ( PDH ) |  |        |  |            |           |
|-----------------------------|--|--------|--|------------|-----------|
| Filling                     |  | trauma |  | extraction | operation |
| Others:                     |  |        |  |            |           |

| Medical History ( MH ) Have you ever had any of the following ? Yes / No |  |                  |  |                     |  |
|--------------------------------------------------------------------------|--|------------------|--|---------------------|--|
| General Health:                                                          |  |                  |  |                     |  |
| High Blood pressure                                                      |  | Diabetes         |  | CVA                 |  |
| Heart Diseases                                                           |  | Thyroid problems |  | Artificial Implants |  |
| Bleeding Disorders                                                       |  | Epilepsy         |  | AIDS / HIV          |  |
| Respiratory Diseases                                                     |  | GIT Diseases     |  | Hepatitis           |  |
| Liver Diseases                                                           |  | Kidney diseases  |  | Allergy             |  |
| Others:                                                                  |  |                  |  |                     |  |

Previous Hospitalization ..... If yes specify .....

Smoking ..... If yes How many ? ..... / Day .

Do you take any drugs ? ( Specify ) ..... Do you have drug sensitivity ? .....

Female patients , are you pregnant ? .....

| Clinical Examination          |             |     |    |            |  |                |                   |              |  |
|-------------------------------|-------------|-----|----|------------|--|----------------|-------------------|--------------|--|
| Extra oral examination (EO)   |             |     |    |            |  |                |                   |              |  |
| Face color                    |             |     |    |            |  |                | Conjunctiva color |              |  |
| Facial symmetry               |             |     |    |            |  |                |                   |              |  |
| swelling                      | Site        |     |    |            |  | Color          |                   |              |  |
|                               | Consistency |     |    |            |  | Tenderness     |                   |              |  |
| Mucocutaneous sinus formation |             | yes | No | Lymph node |  | Palpable       |                   | Not palpable |  |
| Trismus                       |             | yes | No | Clicking   |  | yes            |                   | No           |  |
| Intra oral examination (IO)   |             |     |    |            |  |                |                   |              |  |
| Oral mucosa :                 |             |     |    |            |  | Tongue :       |                   |              |  |
| Gingiva :                     |             |     |    |            |  | Oral hygiene : |                   |              |  |
| swelling                      | Site        |     |    |            |  | color          |                   |              |  |
|                               | consistency |     |    |            |  | tenderness     |                   |              |  |
| Sinus formation               |             | yes | No |            |  |                |                   |              |  |
| Others:                       |             |     |    |            |  |                |                   |              |  |

| Investigations:                   |  |  |  |                       |                     |  |  |  |  |
|-----------------------------------|--|--|--|-----------------------|---------------------|--|--|--|--|
| Intra oral radiograph             |  |  |  | Extra oral radiograph | Lab. Investigations |  |  |  |  |
| Diagnosis:                        |  |  |  |                       |                     |  |  |  |  |
| 8 7 6 5 4 3 2 1   1 2 3 4 5 6 7 8 |  |  |  |                       |                     |  |  |  |  |
| Treatment :                       |  |  |  |                       |                     |  |  |  |  |
| 8 7 6 5 4 3 2 1   1 2 3 4 5 6 7 8 |  |  |  |                       |                     |  |  |  |  |

| Anesthetic technique and instrument selection: |  |  |  |  |  |  |  |  |  |
|------------------------------------------------|--|--|--|--|--|--|--|--|--|
| Surgical technique :                           |  |  |  |  |  |  |  |  |  |
| Instructions and medication:                   |  |  |  |  |  |  |  |  |  |

| Starting | finishing | Mark                           |                            |
|----------|-----------|--------------------------------|----------------------------|
|          |           | Patient preparation (5%)       | Anaesthesia (25%)          |
|          |           | Case sheet documentation (10%) | Instruments selection (5%) |
|          |           | Diagnosis (5%)                 | Extraction (30%)           |
|          |           | Patient position (5%)          | Pos-Op instructions (5%)   |
|          |           | Operator position (5%)         | Chair clearance (5%)       |
|          |           | Total mark                     |                            |

و عليه وقعت

اوافق على معالجتى من قبل الطالب

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# **IN ORAL SURGERY PRACTICE, CLINICIAN IS OFTEN FACED WITH THE DIAGNOSIS OF THE FOLLOWING CONDITIONS:-**

- 1- Dental and facial pain. 2- Swelling (lump, mass).
- 3- Ulcers. 4- Injuries (dental, facial bones).
- 5- Temporomandibular joint problems.
- 6- Medically compromised patient. 7- Facial deformity.



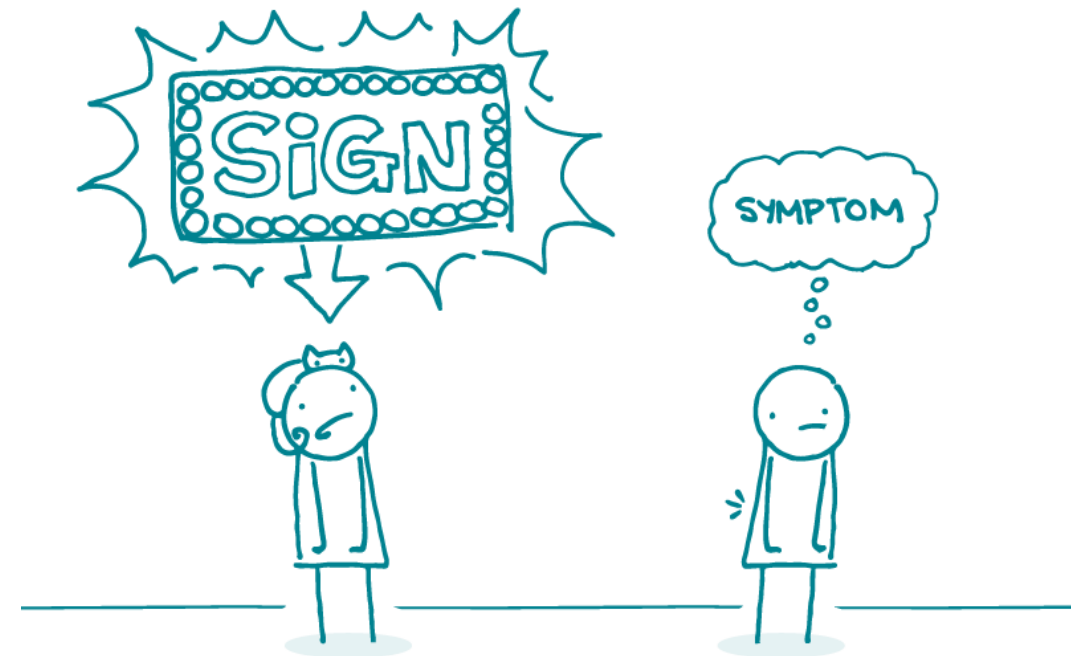
# HISTORY TAKING

The art of taking an accurate case history is probably **the most important single step in the diagnosis of medical or surgical condition**. History taking should be systematic, using special set or sequences. During history taking the clinician or the dental surgeon listen to the patient's story or talks and list the symptoms in order of severity or importance using patient's words.

# HISTORY TAKING

**Symptoms:-** Means a subjective problem that the patient describes e.g. pain, parasthesia.

**Signs:-** Means (objective) an abnormal presentation detectable by the clinician, e.g. swelling, ulcer. So detection of signs and symptoms of a disease may aid in diagnosis of that disease.



# OBJECTIVES OF TAKING HISTORY:-

- 1-To provide dentist with information that is necessary for making diagnosis.
- 2-To establish a good and positive professional relationship with the patient which increase cooperation and confidence.

# OBJECTIVES OF TAKING HISTORY:-

3-To provide the dentist with information concerning patient's **past and present medical, dental and personal history.**

4-To provide information about patient's **systemic health** which may greatly affect the treatment plan and prognosis and diseases that **could be transmitted** to the dentist, his staff or other patients.

# **OBJECTIVES OF TAKING HISTORY:-**

5-It serves as a legal document.

6- For research purposes.



## ▲ How you take history:-

1-During history taking the dentist should encourage his patient to describe his symptoms in his own words, interrupting his story only to explain a point or stop a useless talk. A clear and concise summary of patient's complaints should be recorded in the case sheet. The symptoms should be recorded or being listed in order of its importance (e.g. pain, swelling, bleeding).



## ▲ How you take history:-

2-During taking history give the patient **your whole attention and never take shortcuts.** You have to avoid speed in taking the history, so you have to give the patient a **suitable time** to give all information, because hurry in taking history may lead to many pitfalls that affect the accuracy or completeness.



## ▲ How you take history:-

3-You have to **avoid the leading questions** (e.g. does the pain comes on taking hot or cold?) it's better to ask him what is or what are the things that brings pain to you? Or anything hurt you?

4-During taking history **don't depend on the patient diagnosis or the diagnosis of a previous doctor**, so you have to ask the patient to describe his complaining-only-to establish your diagnosis





## ▲ **Components of the patient history**

The case history may include commonly the following sections or component:

- 1- Biographic data (personal history)**
- 2- Chief complaint. (C.C)**
- 3-History of the chief complaint (history of the present illness). H.P.I**
- 4- Past dental history**
- 5- Medical history and systems review**
- 6- Family history.**

## 1- Biographic data

- ▶ Includes the full name of the patient, age, gender (sex), address, telephone number and occupation. These information may aid or contribute to the diagnosis since some medical problems have a tendency to occur in a particular age group, gender or race. The patient occupation may be associated with a particular disease or may influence the type of therapy.

## 2- Chief complaint:

- ▶ The chief complaint is usually **the reason for the patient's visit**. The chief complaint(s) is best stated in the patient's own words in a brief summary of the problems (e.g. pain, swelling, ulcer, paraesthesia, numbness, clicking, halitosis, bleeding, trismus).
- ▶ If the patient complaining of several symptoms in which case they should be listed, but with the major complaint first.

### 3- History of present illness

- ▶ This part of the story must be gone into complete details and get the patient to **tell** the story in his fashion, never ask the patient leading questions and you have **to see** if the patient in a condition able to give you a history which is reliable and his statement can be relied upon. It's best to start by asking the patient about:-

- 
- 1- Duration (record the length of the complaint).
  - 2- Onset (date of onset, manner of onset).
  - 3- Precipitating/predisposing factors, (e.g. hot, cold, sweet).



4- Characteristic, and this includes:-

- a) Nature e.g. (continuous, intermittent, stabbing).
- b) Severity e.g. (mild, severe, very severe).
- c) Location.
- d) Radiation (feeling of pain in site other than that of causative lesion, called referred pain).



e) Temperature features.

f) Aggravating- factors.

g) Relieving factors.

h) Associated constitutional symptoms and signs.



5- Course and progress.

6- Therapy:-

*a) Type of therapy and dose.*

*b) Provider.*

*c) Effect of therapy.*

*d) Date of therapy.*

7- Other information.



So if a patient comes with a chief complaint (pain) very detailed history of the pain should be taken and particular attention paid to the following points:-

| History of Present Illness (HPI) |       |                 |              |          |                  |  |
|----------------------------------|-------|-----------------|--------------|----------|------------------|--|
| Pain site :                      |       |                 |              |          |                  |  |
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|                                  | sharp |                 | intermittent |          | not tender       |  |
| Pain onset :                     | day   |                 | Night        |          | Non specific     |  |
| Pain duration:                   |       | Trigger factors |              |          | Pain persistence |  |
| Swelling                         |       |                 |              | Duration |                  |  |
| Others:                          |       |                 |              |          |                  |  |

▶ **a) The duration of pain:** Whether any incident which might have played some part in the etiology of the pain precede its onset (e.g. a blow on the jaw, dental treatment), duration record the length of the pain.

**b) Site of the pain:** The patient should be asked to point to the place where the pain is felt, using his finger.

▶ **c) Any radiation of the pain:** If the pain radiates, the patient should be asked to demonstrate its course with the tip of his finger. On other occasions pain maybe felt in a site other than of the causative lesion or remote from the diseased area and this type called "referred pain", e.g. pain of pericoronitis radiates to the ear.

► **d) The precise characteristic of the pain:** the pain maybe described as sharp, severe, dull, throbbing, excruciating, lancinating, mild, continuous, intermitted, all these objectives can be applied to the pain in different pathological process which may help you in the diagnosis. (In acute pulpitis, the pains is sharp and severe, in acute dental abscess the pain is dull, throbbing and severe and the tooth tender, in acute maxillary sinusitis the pain is dull, throbbing and continuous).

**e) Timing of pain:** Some pains are characteristically worse at particular time in the day e.g. pulpal pain often wakens the patient at night and tend to keep him awake, in acute periodontitis the pain is worse at meal time.

**f) Any factors which precipitate the pain:** Pulpal pain is often precipitated by thermal and osmotic stimuli (hot, cold, sweet). Periodontal pain often precipitated by biting and chewing.

g) **factors or Any drugs which relieve pain:** This will give you an idea about the nature and duration of severity of the pain.

h) **The presence of other symptoms:** Like when patient says that, “the pain started two days ago, **then** a swelling appeared after that” or “discharging sinus appeared” or “a discharge of pus” or he/she says: “pain, swelling then parasthesia of the lower lip” ...etc.

▶ **i) The patient also may be asked about relevant past medical history** which may assist you in the diagnosis of the pain like patient with facial pain of vascular origin like migraine, or chronic psychosomatic origin or angina (angina pectoris) pain. In addition to that the patient asked about his opinion of the cause of the pain.

Another example of complaint is:- **Patient presented with a "lump or mass":-**

The oral surgeon must be ascertained by asking some questions: -

- 1- How long the swelling has been present.
- 2- Whether it is getting larger or smaller or fluctuated in size.



3- What are the symptoms of the lump: The lump maybe painful or not. If the lump is associated with Parasthesia or numbness of the lower lip for example.

4- Whether there is any possible cause for the swelling e.g. trauma, injuries, or systemic illness known to the patient.

5- What made the patient notice the lump? By feeling or because it is painful or someone else noticed it and told him.



## 4-Past dental history (P.D.H):-

The past dental history includes:-

- 1- The frequency of previous visits (e.g. previous extractions or oral surgical procedures).
- 2- Any difficulties or complications (e.g. excessive bleeding or fainting).
- 3-Attitude towards previous dental treatment.



4- Determination of the availability of **past dental or oral radiographs**. In other words, it is important to ask the patient about any type of dental or oral treatment received before, and if there is any complications or un satisfaction arise and his impression about the type of treatment.

## 5-Medical history and systems review (M.H):-

The patient's medical history includes review, the past and the present illness or diseases because:-

1- These information (M.H) may aid in the diagnosis of various conditions occurring **or has oral manifestation** that are related to specific systemic disease (e.g. aids, leukemia).



2- The presence of many diseases may lead or need **modification for the treatment plan**, and affect the manner in which therapy is provided.

3- **Drugs used in treatment of some systemic diseases** can also have effects on the mouth (have oral manifestation), or dictate some modification to the dental or surgical treatment (e.g. anticoagulant drugs, chemotherapy).



## **The past medical history includes:-**

- 1- Previous serious illness or diseases.
- 2- Childhood diseases.
- 3- Hospitalization.
- 4- Operations.
- 5- Injuries to the head and neck.
- 6- Allergy to drugs or general allergy.
- 7- Listing of medication taken in the last six months.



## Some examples of serious illness:-

- ◆ Heart attack or diseases (e.g. myocardial infarction, angina pectoris).
- ◆ Stroke (cerebrovascular accident C.V.A).
- ◆ Hypertension.
- ◆ Heart failure.
- ◆ Bleeding disorders.
- ◆ Diabetes.
- ◆ Rheumatic fever or disease. ◆ Hospitalizations may indicate past disease and how it was treated.

- 
- ◆ Aids (acquired immune-deficiency syndrome).
  - ◆ Viral hepatitis.
  - ◆ Neoplasm and the method of treatment (surgical, cytotoxic drugs) especially if the growth in the head and neck region **or** previous radiation (radiotherapy).
  - ◆ Allergic reaction to drugs.





The review of systems includes:- Cardiovascular system, respiratory system, central nervous system, genitourinary system, musculoskeletal system, endocrine system, ears, eye, vital signs (blood pressure, pulse, temperature, respiratory rate).



# Components of medical history

Any patient come to you should be asked certain concise questions that aid you to have medical history, and this includes:

- 
1. If he is currently receiving any medical care or under supervision of any clinician.
  - 2 . Whether he has been hospitalized and Why?
  - 3 . If you have any serious illness remembered by the patient?
  - 4 . If you have any surgical operation before?
  - 5 . If your patient takes any type of drugs before in the past or present time?



## Family History: (F. H.)

Details of (F.H.) may reveal valuable information about diseases that are occurring in families (e.g. Tuberculosis, Hemophilia, Psychiatric or neurotic disorders, Breast cancer) Congenital Anomalies such as lip clefts or palate clefts.



**ANY**  
**QUESTIONS?**



جامعة الكفيل  
University Of Alkafeel

# Thank you